



OPERATIVE NOTE

Department of General Surgery · OT-2

Patient Name: Rajesh Sharma

Age / Sex: 46 Y / Male

Date of Surgery: 13-01-2026

Surgeon: Dr. Anil Mehta, MS (General Surgery)

Anaesthetist: Dr. Priya Nair, MD (Anaesthesiology)

Scrub Nurse: Sr. Leena Pillai

UHID: UHID-123456

IPD No.: IPD/2026/004512

Operation Time: 10:15 - 11:25

Assistant Surgeon: Dr. Rohan Das

Anaesthesia: General anaesthesia

Wound Class: Clean-contaminated

Pre-operative Diagnosis: Acute cholecystitis

Post-operative Diagnosis: Acute cholecystitis

Procedure Performed: Laparoscopic Cholecystectomy

OPERATIVE FINDINGS

- Distended, inflamed gall bladder with a thickened wall and multiple calculi.
- Omental adhesions to the fundus of the gall bladder; no perforation or pus.
- Cystic duct and cystic artery clearly identified; common bile duct normal.
- Liver, stomach and duodenum grossly normal.

PROCEDURE DETAILS

- Patient supine under general anaesthesia with endotracheal intubation; parts painted and draped.
- Pneumoperitoneum created by the open (Hasson) technique at the umbilicus; four-port technique used.
- Omental adhesions released by blunt and harmonic dissection.
- Calot's triangle dissected and the critical view of safety achieved.
- Cystic duct and cystic artery secured with Hem-o-lok polymer ligating clips (6 clips applied in total) and divided.
- Gall bladder separated from the liver bed with hook diathermy and retrieved in an endobag through the epigastric port.
- Haemostasis confirmed; liver bed irrigated; 16 Fr subhepatic drain placed through the right lateral port.
- Ports closed under vision; umbilical fascia closed; skin closed with absorbable sutures.



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IMPLANTS USED

Implant	Qty	Lot No.	Placement
Hem-o-lok Polymer Ligating Clips (ML)	6	HL-44710	Cystic duct (3), cystic artery (3)

SUMMARY

Specimen: Gall bladder with calculi, sent for histopathology
Drain: 16 Fr subhepatic drain
Blood Transfusion: Not required
Estimated Blood Loss: 30 ml
Complications: None
Condition: Stable; shifted to recovery at 11:40

POST-OPERATIVE INSTRUCTIONS

- Nil by mouth for 6 hours, then sips of water; soft diet from post-operative day 1.
- Continue Inj. Ceftriaxone 1 g IV 12-hourly and Inj. Metronidazole 500 mg IV 8-hourly.
- Analgesia as per chart; monitor vitals, drain output and urine output.
- Early ambulation; review the drain on post-operative day 2.

DR. ANIL MEHTA
MS (GENERAL SURGERY)
REG. DEMO-MC-20417

Operating Surgeon

Dr. Anil Mehta, MS (General Surgery) · Reg. No. DEMO-MC-20417
Date: 13-01-2026